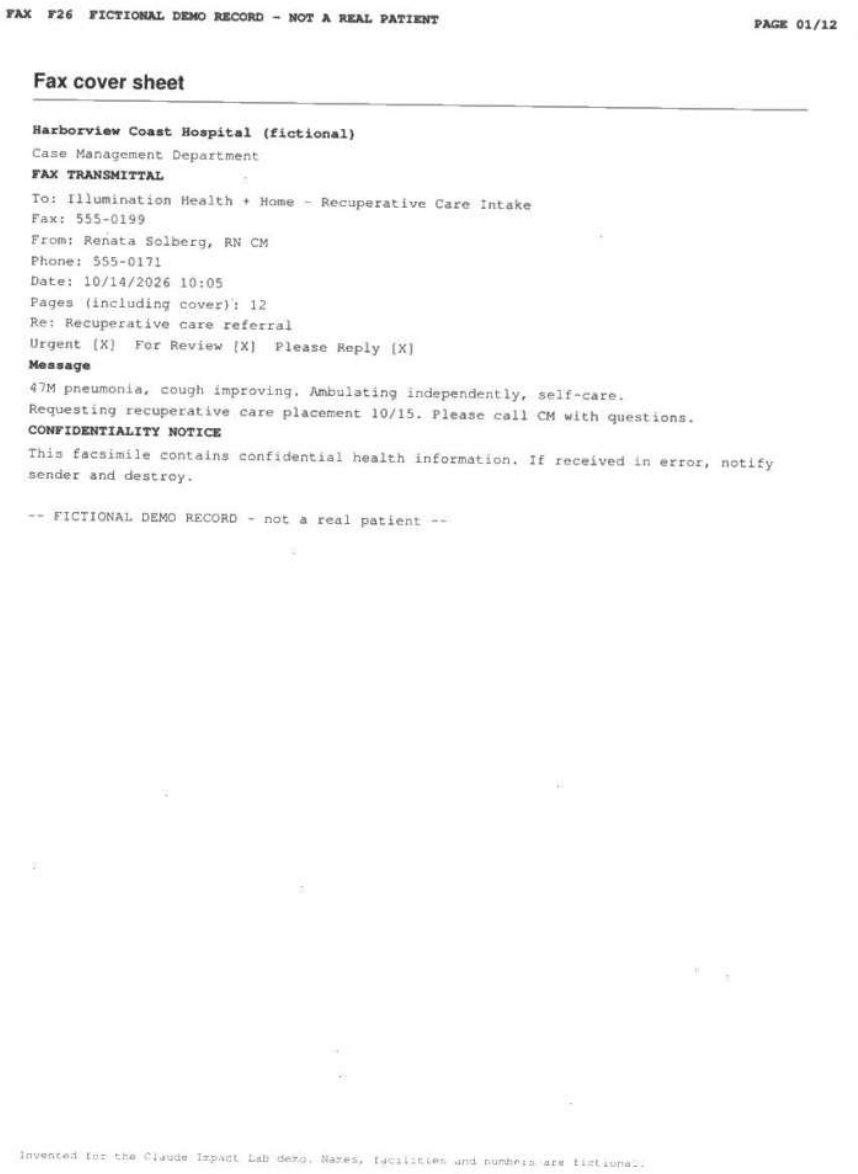
Referral packet F26: readable copy

Source: F26.pdf (fictional), 12 pages. Left side of each sheet is the original page, right side is a typed transcript. Black boxes are privacy redactions, not missing information.

Page map

Page	Document	Type	Legibility	Check by hand
1	Fax cover sheet	fax_cover	good	
2	IHH Recuperative Care Referral Form	referral_form	good	
3	CalAIM Community Supports authorization request	calaim_auth	good	
4	Patient face sheet	face_sheet	good	
5	History and Physical	h_and_p	good	
6	Hospitalist progress note	progress_note	good	
7	Infectious disease consult note	id_consult	good	
8	Physical therapy evaluation	pt_eval	good	
9	Nursing shift note	nursing_note	good	
10	Social work note	social_work	good	
11	Labs, imaging and vitals	labs	good	
12	Current medication list	med_list	good	



FAX F26 FICTIONAL DEMO RECORD - NOT A REAL PATIENT PAGE 01/12

Fax cover sheet

Harborview Coast Hospital (fictional)
Case Management Department
FAX TRANSMITTAL
To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Renata Solberg, RN CM
Phone: 555-0171
Date: 10/14/2026 10:05
Pages (including cover): 12
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]
Message
47M pneumonia, cough improving. Ambulating independently, self-care.
Requesting recuperative care placement 10/15. Please call CM with questions.
CONFIDENTIALITY NOTICE
This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 1: Fax cover sheet

fax_cover | orientation normal | legibility good

Harborview Coast Hospital (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Renata Solberg, RN CM
Phone: 555-0171
Date: 10/14/2026 10:05
Pages (including cover): 12
Re: Recuperative care referral
Urgent **[X]** For Review **[X]** Please Reply **[X]**

Message

47M pneumonia, cough improving. Ambulating independently, self-care.
Requesting recuperative care placement 10/15. Please call CM with questions.

CONFIDENTIALITY NOTICE

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-- FICTIONAL DEMO RECORD - not a real patient --

FAX F26 FICTIONAL DEMO RECORD - NOT A REAL PATIENT PAGE 02/12

ILLUMINATION Recuperative Care Referral Form

HEALTH • HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ilhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Harborview Coast Hospital (fictional) Dept/Floor: 5 West Medical
SW/CM/RN: Renata Solberg, RN CM Phone/Pager #: 555-0171
Email: rsolberg@harborview.example Nursing Station #: 555-0172
Authorized by: Dr. L. Abernathy Phone #:

Patient Information

Patient Name: Marcus Doe MRN: HC-4418207 Date of Birth (dd/mm/yyy): 07/22/1979
Gender: ☒ Male ☐ Female ☐ Transgender ☐ Other Ethnicity: Hispanic or Latino
Last 4 # of Social Security: xxx-xx- 0000 Insurance: Orange Coast Health Plan (fictional) Medi-Cal Preferred Language: English, Spanish
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
Pneumonia, right upper lobe; weight loss
Admit Date: 10/09/2026 Any Wounds? ☐ Yes ☒ No
If yes, explain the number, location, size and stage:

Independent with Wound Care? ☒ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☒ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 10/09/2026 Results: Chest X-ray negative

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	Yes* ✕ No	Active Infection	✕ Yes* No
*Explain		*Explain pneumonia	
Self Administer All Meds	✕ Yes No*	Ambulatory	✕ Yes No
Continent of Bowel & Bladder	✕ Yes No	Assistive Device	Yes* ✕ No
Colostomy/Ileostomy	Yes ✕ No	*Explain	
Foley Catheter	Yes ✕ No	Hospice	Yes* ✕ No
Diabetes	Yes ✕ No	*Explain	
Insulin	Yes ✕ No	Car	Yes ✕ No
Diabetes related Oral Meds	Yes ✕ No	Service Animal	Yes ✕ No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? ✕ No Yes

☐ Mental Health DX If checked, what type?
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain None

Length of Stay Authorized in Recupertive Care: 14 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

Page 2: IHH Recuperative Care Referral Form

referral_form | orientation normal | legibility good

ILLUMINATION HEALTH + HOME

Recuperative Care Referral Form (RC0725-SH)

Referring Party

Hospital: Harborview Coast Hospital (fictional)
Department/Floor: 5 West Medical
Social Worker/Case Manager/Nurse: Renata Solberg, RN CM
Phone: 555-0171
Email: rsolberg@harborview.example
Nursing Station: 555-0172
Authorized By: Dr. L. Abernathy

Patient Information

Name: Marcus Doe
MRN: HC-4418207
Birth Date: 07/22/1979
Gender: M
Ethnicity: Hispanic or Latino
SSN (last 4): 0000
Insurance: Orange Coast Health Plan (fictional) Medi-Cal
Preferred Language: English, Spanish
Housing Situation:
Currently Unhoused **[X]**
4+ episodes of homelessness in the past 3 years [ ]

Hospital Admission Details

Chief Complaint/Admitting Diagnosis: Pneumonia, right upper lobe; weight loss
Admit Date: 10/09/2026
Any Wounds: Yes [ ] No **[X]**
Number/Location/Size/Stage:
Independent with Wound Care: Yes [ ] No [ ]
Home Health Ordered: Yes [ ] No **[X]**
PPD/TB Test or Chest X-Ray: Date: 10/09/2026 Result: Chest X-ray negative

History of Health & Living Situation

O2: Yes [ ] No **[X]**
Self Administer All Meds: Yes **[X]** No [ ]
Continent of Bowel & Bladder: Yes **[X]** No [ ]
Colostomy/Ileostomy: Yes [ ] No **[X]**
Foley Catheter: Yes [ ] No **[X]**
Diabetes: Yes [ ] No **[X]**
Insulin: Yes [ ] No **[X]**
Diabetes Oral Meds: Yes [ ] No **[X]**
Active Infection: Yes **[X]** No [ ] pneumonia
Ambulatory: Yes **[X]** No [ ]
Assistive Device: Yes [ ] No **[X]**
Hospice: Yes [ ] No **[X]**
Car: Yes [ ] No **[X]**
Service Animal: Yes [ ] No **[X]**

Behavioral and Mental Health Status

Any limitations or challenges: No **[X]** Yes [ ]
Mental Health DX: Yes [ ] No **[X]** Type:
Non-compliant: Yes [ ] No **[X]**
Cognitive Impairment: Yes [ ] No **[X]**
Forgetful: Yes [ ] No **[X]**
Registered Sex Offender: Yes [ ] No **[X]**
Other: None

Length of Stay Authorized

Days: 14

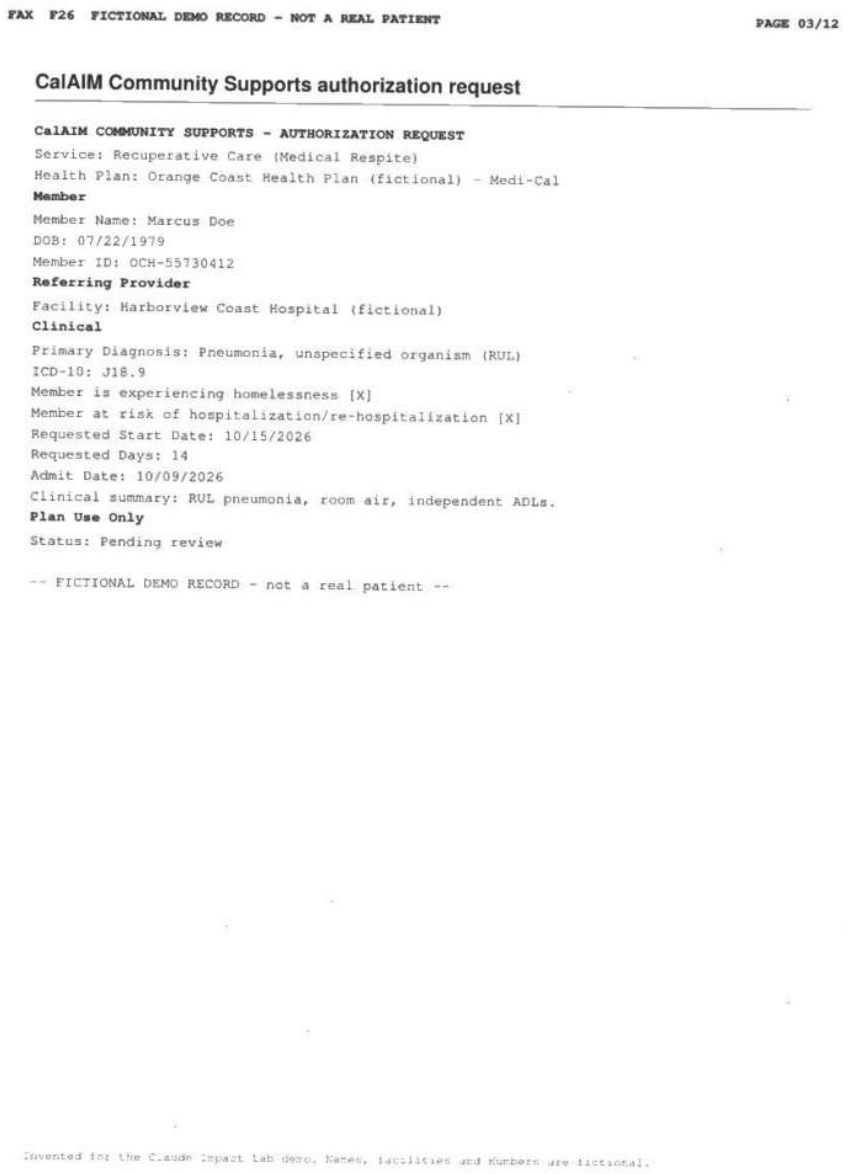
Illumination Staff Only

Date Received:
Reviewed By:
Approved: Yes [ ] No [ ]
Date:
Authorization #:
Reason for Denial:
Admission Date/Time:
Exit Date:
Extension Request: Yes [ ] No [ ]

Referring Party Comments

PPD/TB: chest X-ray negative.
Independent with ADLs, walks in room. Needs safe place to recover.

-- FICTIONAL DEMO RECORD - not a real patient --



Page 3: CalAIM Community Supports authorization request

calaim_auth | orientation normal | legibility good

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Marcus Doe
DOB: 07/22/1979
Member ID: OCH-55730412

Referring Provider

Facility: Harborview Coast Hospital (fictional)

Clinical

Primary Diagnosis: Pneumonia, unspecified organism (RUL)
ICD-10: J18.9
Member is experiencing homelessness **[X]**
Member at risk of hospitalization/re-hospitalization **[X]**
Requested Start Date: 10/15/2026
Requested Days: 14
Admit Date: 10/09/2026
Clinical summary: RUL pneumonia, room air, independent ADLs.

Plan Use Only

Status: Pending review

-- FICTIONAL DEMO RECORD - not a real patient --

Patient face sheet

Harborview Coast Hospital (fictional) - PATIENT FACE SHEET
Patient: Marcus Doe
MRN: HC-4418207
Account #: 99418207
DOB: 07/22/1979
Sex: Male
Preferred Language: English
Ethnicity: Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)
Encounter
Admit Date/Time: 10/09/2026 16:20 (ED arrival 10/09/2026 11:55)
Admit Type: Inpatient
Unit/Bed: 5W-512 (negative pressure room)
Attending: Lorraine Abernathy, MD
Insurance
Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self
Emergency Contact
None listed

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Input Lab demo. Names, facilities and numbers are fictional.

Page 4: Patient face sheet

face_sheet | orientation normal | legibility good

Harborview Coast Hospital (fictional) - PATIENT FACE SHEET

Patient: Marcus Doe
MRN: HC-4418207
Account #: 99418207
DOB: 07/22/1979
Sex: Male
Preferred Language: English
Ethnicity: Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 10/09/2026 16:20 (ED arrival 10/09/2026 11:55)
Admit Type: Inpatient
Unit/Bed: 5W-512 (negative pressure room)
Attending: Lorraine Abernathy, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical

HISTORY AND PHYSICAL
Date: 10/09/2026
CC
Cough x 6 weeks, night sweats, weight loss
HPI
47 y/o M, unhoused x 4 yrs (stays in congregate shelters and outdoors), presents with productive cough x 6 weeks, intermittent blood-streaked sputum, drenching night sweats and about 15 lb unintentional weight loss. Reports a shelter contact with a "TB scare" earlier this year. No prior TB testing on file.
PMH
- Tobacco use
- No known HIV testing on file
Meds PTA
None
Allergies
NKDA
Social
Tobacco 1/2 ppd. EtOH socially, last drink 2 weeks ago. Denies drug use.
Exam
T 100.6F HR 98 BP 126/80 RR 18 SpO2 96% RA. Coarse breath sounds RUL. Thin. AOx4.
Assessment/Plan
1. RUL cavitary lesion with chronic cough, night sweats, weight loss - suspected pulmonary tuberculosis. Airborne isolation in negative pressure room. Sputum AFB smear and culture x3, TB PCR. HIV, QuantiFERON. Infectious Disease consult.
2. Tobacco use - nicotine patch
3. Dispo - SW/CM consult (unhoused). No discharge until ID clears isolation.
Signed: Lorraine Abernathy, MD 10/09/2026 18:30

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 5: History and Physical

h_and_p | orientation normal | legibility good

HISTORY AND PHYSICAL

Date: 10/09/2026

CC

Cough x 6 weeks, night sweats, weight loss

HPI

47 y/o M, unhoused x 4 yrs (stays in congregate shelters and outdoors), presents with productive cough x 6 weeks, intermittent blood-streaked sputum, drenching night sweats and about 15 lb unintentional weight loss. Reports a shelter contact with a "TB scare" earlier this year. No prior TB testing on file.

PMH

- Tobacco use
- No known HIV testing on file

Meds PTA

None

Allergies

NKDA

Social

Tobacco 1/2 ppd. EtOH socially, last drink 2 weeks ago. Denies drug use.

Exam

T 100.6F HR 98 BP 126/80 RR 18 SpO2 96% RA. Coarse breath sounds RUL. Thin. AOx4.

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3. Dispo - SW/CM consult (unhoused). No discharge until ID clears isolation.
Signed: Lorraine Abernathy, MD 10/09/2026 18:30

-- FICTIONAL DEMO RECORD - not a real patient --

Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#5
Date: 10/14/2026 07:50
S: Cough persists, less sputum. Night sweats improving. Eating better.
O: T 99.4F HR 88 BP 122/76 RR 16 SpO2 96% RA. Coarse RUL breath sounds. AOx4.
A/P:
1. Suspected pulmonary TB, RUL cavitary lesion - remains on AIRBORNE ISOLATION. Sputum AFB smear #1 negative, smears #2 and #3 pending. TB PCR pending. ID following; ID has NOT cleared isolation.
2. Empiric RIPE therapy started 10/12 per ID (rifampin, isoniazid, pyrazinamide, ethambutol) with pyridoxine.
3. Dispo - not medically ready for congregate setting until isolation cleared by ID and public health. CM aware.
Signed: Lorraine Abernathy, MD

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 6: Hospitalist progress note

progress_note | orientation normal | legibility good

HOSPITALIST PROGRESS NOTE - HD#5

Date: 10/14/2026 07:50
S: Cough persists, less sputum. Night sweats improving. Eating better.
O: T 99.4F HR 88 BP 122/76 RR 16 SpO2 96% RA. Coarse RUL breath sounds. AOx4.
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2. Empiric RIPE therapy started 10/12 per ID (rifampin, isoniazid, pyrazinamide, ethambutol) with pyridoxine.
3. Dispo - not medically ready for congregate setting until isolation cleared by ID and public health. CM aware.
Signed: Lorraine Abernathy, MD

-- FICTIONAL DEMO RECORD - not a real patient --

Infectious disease consult note

INFECTIOUS DISEASE CONSULT - FOLLOW-UP
Date: 10/13/2026 15:10
Imaging reviewed: RUL thick-walled cavitary lesion with surrounding nodular opacities, highly concerning for active pulmonary TB.
Sputum AFB smear #1 negative 10/11. Smears #2, #3 and TB PCR pending. Cultures take up to 6-8 weeks.
Recommendations:
- Continue airborne isolation. NOT cleared for discharge to congregate or shared living setting.
- Isolation may be reconsidered only after 3 negative AFB smears and negative TB PCR, or after adequate treatment with clinical response, per ID and County Public Health.
- Continue RIPE + pyridoxine; directly observed therapy (DOT) referral to County TB clinic.
Signed: N. Vasquez-Hale, MD (Infectious Disease)

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 7: Infectious disease consult note

id_consult | orientation normal | legibility good

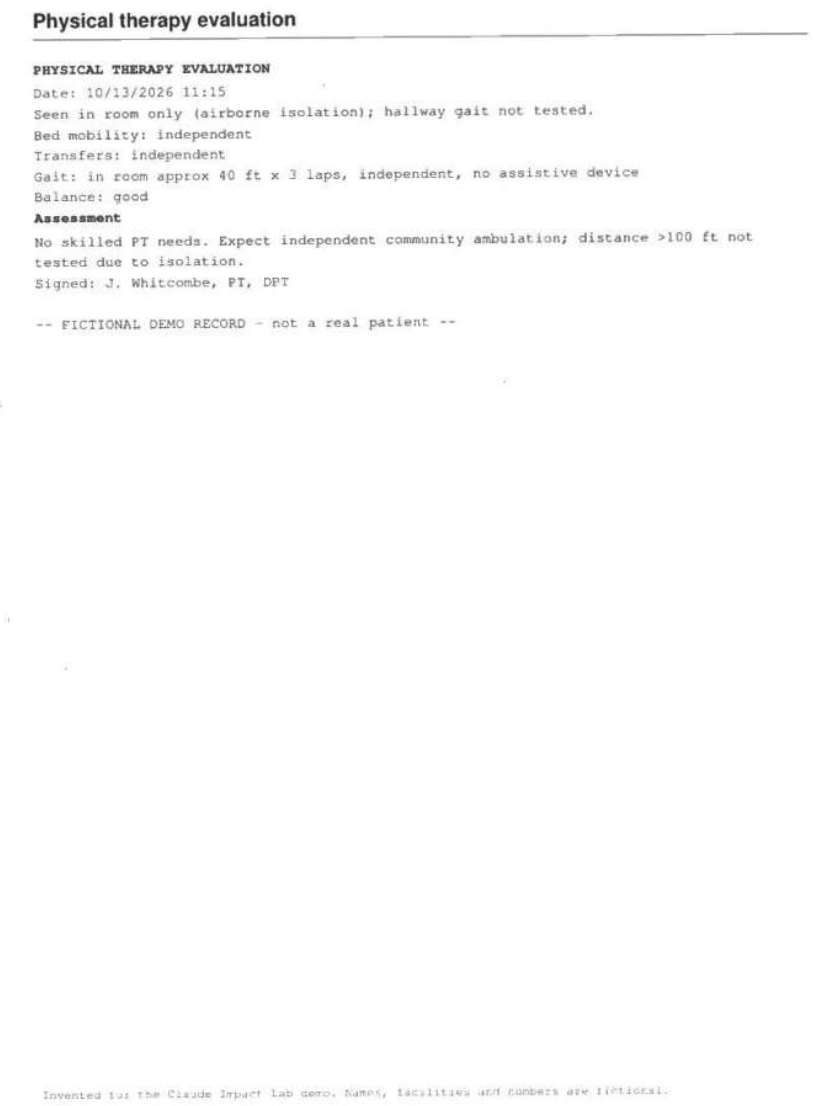
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Recommendations:

- Continue airborne isolation. NOT cleared for discharge to congregate or shared living setting.
- Isolation may be reconsidered only after 3 negative AFB smears and negative TB PCR, or after adequate treatment with clinical response, per ID and County Public Health.
- Continue RIPE + pyridoxine; directly observed therapy (DOT) referral to County TB clinic.

Signed: N. Vasquez-Hale, MD (Infectious Disease)

-- FICTIONAL DEMO RECORD - not a real patient --



Physical therapy evaluation

PHYSICAL THERAPY EVALUATION
Date: 10/13/2026 11:15
Seen in room only (airborne isolation); hallway gait not tested.
Bed mobility: independent
Transfers: independent
Gait: in room approx 40 ft x 3 laps, independent, no assistive device
Balance: good
Assessment
No skilled PT needs. Expect independent community ambulation; distance >100 ft not tested due to isolation.
Signed: J. Whitcombe, PT, DPT

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 8: Physical therapy evaluation

pt_eval | orientation normal | legibility good

PHYSICAL THERAPY EVALUATION

Date: 10/13/2026 11:15
Seen in room only (airborne isolation); hallway gait not tested.
Bed mobility: independent
Transfers: independent
Gait: in room approx 40 ft x 3 laps, independent, no assistive device
Balance: good

Assessment

No skilled PT needs. Expect independent community ambulation; distance >100 ft not tested due to isolation.
Signed: J. Whitcombe, PT, DPT

-- FICTIONAL DEMO RECORD - not a real patient --

Nursing shift note

NURSING SHIFT NOTE
Date: 10/14/2026 06:15
Pt AOx4. Independent with ADLs, showers independently in room bathroom. Continent of bowel and bladder.
Takes RIPE meds with RN direct observation; states names and purpose of each med.
Calm and cooperative. Wears surgical mask when staff in room.
Isolation status: AIRBORNE - negative pressure room, N95 required for staff.
Signed: P. Delacroix, RN

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 9: Nursing shift note

nursing_note | orientation normal | legibility good

NURSING SHIFT NOTE

Date: 10/14/2026 06:15
Pt AOx4. Independent with ADLs, showers independently in room bathroom. Continent of bowel and bladder.
Takes RIPE meds with RN direct observation; states names and purpose of each med.
Calm and cooperative. Wears surgical mask when staff in room.
Isolation status: AIRBORNE - negative pressure room, N95 required for staff.
Signed: P. Delacroix, RN

-- FICTIONAL DEMO RECORD - not a real patient --



Social work note

SOCIAL WORK NOTE
Date: 10/13/2026 14:00
Pt pleasant and engaged, AOx4. Denies SI/HI. Denies hallucinations. No psychiatric history.
No conservatorship. Not enrolled in FSP.
Unhoused x 4 yrs; stays in congregate shelters. Public health notified of suspected TB for contact investigation.
Signed: Renata Solberg, RN CM / E. Thorsen, MSW

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 10: Social work note

social_work | orientation normal | legibility good

SOCIAL WORK NOTE

Date: 10/13/2026 14:00
Pt pleasant and engaged, AOx4. Denies SI/HI. Denies hallucinations. No psychiatric history.
No conservatorship. Not enrolled in FSP.
Unhoused x 4 yrs; stays in congregate shelters. Public health notified of suspected TB for contact investigation.
Signed: Renata Solberg, RN CM / E. Thorsen, MSW

-- FICTIONAL DEMO RECORD - not a real patient --

Labs, imaging and vitals

LABS

10/09 | WBC 10.4 | Hgb 12.1 L | Plt 388 | Na 135 | K 4.1 | Cr 0.8 | Albumin 3.1 L
10/09 HIV 1/2 Ag/Ab: Nonreactive
10/10 QuantiFERON-TB Gold: Positive
10/11 Sputum AFB smear #1: Negative (no AFB seen)
10/12 Sputum AFB smear #2: Pending
10/13 Sputum AFB smear #3: Pending
10/11 MTB PCR (sputum): Pending
10/09 MRSA nares PCR: Negative
10/14 05:30 SARS-CoV-2 rapid antigen: Negative

RADIOLOGY REPORT

10/09 CXR PA/lat: Right upper lobe cavitary lesion, 3.2 cm, with surrounding nodular opacities. No effusion. No pneumothorax.
Impression: Findings concerning for active pulmonary tuberculosis. Recommend airborne isolation.
10/10 CT chest: RUL thick-walled cavity with tree-in-bud nodularity. No pulmonary embolism.

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
10/13 08:00	99.1	90	124/78	16	96%	RA
10/13 14:00	99.6	92	126/80	18	96%	RA
10/13 20:00	99.2	88	120/74	16	95%	RA
10/14 02:00	98.8	84	118/72	16	96%	RA
10/14 07:50	99.4	88	122/76	16	96%	RA

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 11: Labs, imaging and vitals

labs | orientation normal | legibility good

LABS

10/09 | WBC 10.4 | Hgb 12.1 L | Plt 388 | Na 135 | K 4.1 | Cr 0.8 | Albumin 3.1 L
10/09 HIV 1/2 Ag/Ab: Nonreactive
10/10 QuantiFERON-TB Gold: Positive
10/11 Sputum AFB smear #1: Negative (no AFB seen)
10/12 Sputum AFB smear #2: Pending
10/13 Sputum AFB smear #3: Pending
10/11 MTB PCR (sputum): Pending
10/09 MRSA nares PCR: Negative
10/14 05:30 SARS-CoV-2 rapid antigen: Negative

RADIOLOGY REPORT

10/09 CXR PA/lat: Right upper lobe cavitary lesion, 3.2 cm, with surrounding nodular opacities. No effusion. No pneumothorax.
Impression: Findings concerning for active pulmonary tuberculosis. Recommend airborne isolation.
10/10 CT chest: RUL thick-walled cavity with tree-in-bud nodularity. No pulmonary embolism.

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
10/13 08:00 | 99.1 | 90 | 124/78 | 16 | 96% | RA
10/13 14:00 | 99.6 | 92 | 126/80 | 18 | 96% | RA
10/13 20:00 | 99.2 | 88 | 120/74 | 16 | 95% | RA
10/14 02:00 | 98.8 | 84 | 118/72 | 16 | 96% | RA
10/14 07:50 | 99.4 | 88 | 122/76 | 16 | 96% | RA

-- FICTIONAL DEMO RECORD - not a real patient --

Current medication list

CURRENT MEDICATIONS
Patient: Marcus Doe MRN: HC-4418207
- Rifampin 600 mg PO daily (DOT)
- Isoniazid 300 mg PO daily (DOT)
- Pyrazinamide 1500 mg PO daily (DOT)
- Ethambutol 1200 mg PO daily (DOT)
- Pyridoxine 50 mg PO daily
- Nicotine patch 14 mg/24h daily
No IV medications. No home O2.
Discharge medication list: not yet prepared

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact lab demo. Names, facilities and numbers are fictional.

Page 12: Current medication list

med_list | orientation normal | legibility good

CURRENT MEDICATIONS

Patient: Marcus Doe MRN: HC-4418207

- Rifampin 600 mg PO daily (DOT)
- Isoniazid 300 mg PO daily (DOT)
- Pyrazinamide 1500 mg PO daily (DOT)
- Ethambutol 1200 mg PO daily (DOT)
- Pyridoxine 50 mg PO daily
- Nicotine patch 14 mg/24h daily

No IV medications. No home O2.
Discharge medication list: not yet prepared

-- FICTIONAL DEMO RECORD - not a real patient --